14.1 Hypovolemic Shock

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I feel cold and thirsty." **Objective** • BP 80/50 mmHg, HR 132/min • Skin cold and clammy; capillary refill 4 seconds • Urine output 10 mL/hour • Known to have lost blood or fluid	**ACTUAL** **Deficient fluid volume** related to acute blood or fluid loss as evidenced by hypotension, tachycardia, poor perfusion and oliguria.	Within 2 hours, MAP 65 mmHg or more and urine output at least 0.5 mL/kg/hour.	• Call for help and stop any external bleeding with direct pressure. • Put in two large-bore IV cannulas. • Give warmed fluids and blood products as prescribed. • Check BP, HR, SpO_2 and level of consciousness every 5–15 minutes. • Put in a urinary catheter and record the output every hour.	MAP and urine output on target: **goal met**. If not, get ready for surgery to control the bleeding.
Subjective • Drowsy **Objective** • Confused and slow to respond • SpO_2 92%	**ACTUAL** **Acute confusion** related to reduced cerebral perfusion as evidenced by drowsiness and slow responses.	Alert and oriented as perfusion improves.	• GCS every 15–30 minutes. • Give oxygen to keep SpO_2 on target. • Unless there is a reason not to, keep him flat with his legs up.	Alert: **goal met**.
Subjective • *No specific complaint* **Objective** • Receiving a massive transfusion • Given cold fluids • Uncovered during resuscitation	**RISK** **Risk for hypothermia** as evidenced by risk factors: blood loss, cold IV fluids and exposure during resuscitation.	Core temperature stays above 36 °C.	• Run blood and fluids through a fluid warmer. • Cover him with warm blankets or a forced-air warmer. • Temperature every 30–60 minutes.	Temperature kept normal: **goal met**.